



Echocardiography Report on 12/09/2025

Doctor DIZNABI Ardeshir born on 16/08/1970 55 year - LPU19187
27 Amberley Drive, United Kingdom Hove BN3 8JP

PIN: LPU19187

Clinical Indication: Heart check-up

This ultrasound examination was performed with the patient's verbal consent.

Past Medical and Medication History:

Today in the clinic, there were no clinical signs of active cardiac disease.

LVEDD(cm) 4.46	AORTA	
LVESD(cm) 3.41	Annulus(cm) :	
IVSd(cm) 0.93	SOV(cm) : 3.22	
PWT(cm) 0.9	STJ(cm) : 2.13	
LVEDV(cc) 112	Asc(cm) : 2.7	
LVESV(cc) 43	Ao arch: 2.34	
LVEF (%) 61(Simpson)	LVOT VTI(cm) :	
LV EF: (%) (mmode)	Ao Vmax : 0.83	
TAPSE(cm) 2.76	Ao MPG(mmHg) :	
S' (TV) 0.10		
Mid RV(cm) 3.12	TV/PV	
LA : cm/area: 4.07 /13.68	TR vel (m/sec) :1.5	PV PGmax:
RA area(cm ²): 14.02	PAP(mmHg) :	PI PG:
MV	RVOT VTI(cm)	
E Vel. (m/s) 0.47	IVC diameter (cm) 1.1	
A Vel.(m/s) 0.54	Diastolic Function	
DT(msec)	E _m (m/s) 0.06	
PHT(msec)	A _m (m/s) 0.11	
MPG(mmHg)	E/E _m	
MVA(cm ²)	S _m (cm/s) 0.07	

Study quality/Patient status/HR/ Rhythm: Technically fair/ Calm / 65/ Normal sinus rhythm

Left Ventricle:

-Normal LV cavity size and wall dimensions. Systolic function is expected; no regional wall motion abnormality was detected at rest. (LVEF: 61% by biplane Simpson's). Wall thickness was standard.

Left Atrium :

Based on volumetric assessment, the left atrium is normal in size and has a normal appearance.

Septum:

The inter-atrial and inter-ventricular septa are intact. There is no obvious shunt with colour flow imaging. It is not possible to rule out PFO with TTE alone.

LV Diastolic Function:

Normal LV filling pressure with LV diastolic function. E/A: 0.9, E/Ea: 8

Mitral Valve:

The mitral valve is normal for age, with thin, mobile and billowing leaflets and good excursions. There is a normal annulus and sub-valvular apparatus. There is mild mitral regurgitation.

Aortic Valve:

The aortic valve has three cusps and appears structurally and functionally normal for age, with good cusp separation. No obvious aortic regurgitation is noted.

Aorta:

The aortic root, ascending aorta, and aortic arch appear normal in size, with no obvious abnormalities.

Right ventricle:

The right ventricle is of normal size. Systolic function is normal. The tricuspid annular plane excursion is normal.

Right atrium:

The right atrium is of normal size, as determined by volumetric assessment, and exhibits a normal appearance.

IVC/hepatic veins:

The inferior vena cava is of normal size. There is normal inspiratory collapse.

Tricuspid valve:

The tricuspid valve has thin and mobile leaflets and appears structurally normal. Trace tricuspid valve regurgitation is

present. There is an insufficient TR velocity to estimate PASP, although no obvious features suggest elevated pulmonary pressures. The pulmonary pressure is within normal limits.

Pulmonary valve:

The pulmonary valve is structurally normal with thin and mobile pulmonary valve leaflets. There is physiologic pulmonary regurgitation.

Pulmonary artery:

The main pulmonary artery appears normal in size.

Pericardium:

The pericardium is normal with no pericardial effusion

Others:

No obvious thrombi or masses were detected in this study.

Summary:

LV: Normal LV size and normal systolic function (LV EF: 61%)

Diastole: Normal LV diastolic function

RV: Normal RV size and systolic function

Valves: No significant valve abnormalities, mild MVP and mild MR

PAP: Normal PAP (PASP: 15-18 mmHg)

No PE, Mass or thrombus.

Conclusion:

The trans-thoracic Echocardiogram data are unremarkable.

Recommendation:

Please share this report with your GP or specialist.

Echocardiogram performed and reported by:

FARVEH VAKILIAN, M.D.

ESC core cardiology certificate

ESC Heart Failure certificate

GMC number: 7862164

LPURM1

















